

Q What different types of pain relief are available, and when can I use them?

Some women manage using mainly natural methods of pain relief, such as breathing, for much of labor. For others, effective methods of pain relief may change as labor progresses and intensifies, and many women choose to supplement natural techniques with medical forms of pain relief.

WHAT PAIN RELIEF YOU MIGHT USE

The table below charts how different types of pain relief are commonly used during labor. The delivery of the placenta in the third stage is commonly managed without pain relief.

STAGE	HOW THE PAIN MIGHT FEEL	NATURAL PAIN RELIEF OPTIONS	MEDICAL PAIN RELIEF OPTIONS
1st stage of labor—latent phase	Contractions may stop and start, and can feel like severe menstrual pain, and there may be lower back pain. Pain is usually bearable, such that you can continue to move around and talk during contractions. Contractions gradually get closer together and stronger.	» Breathing: this helps you focus during contractions. » Meditation: can keep you calm and focused. » Water: warm water is soothing and relaxing early on. » Massage: can ease backache and promote relaxation. » TENS: takes the edge off contractions and relieves backache. » Acupuncture: this can help you relax in early labor.	» Analgesics: Occasionally, opioid analgesics may be used to help manage your stronger contractions now.
1st stage of labor—active phase	Contractions are regular, frequent, and intense: you're unable to talk or move through them. There may be nausea, strong pressure in the back, and pain may radiate down the legs.	» Rhythmic breathing: controlled breathing helps you focus and maximizes oxygen flow to the muscles. » Meditation/massage: these continue to be helpful. » Water: supportive and relieves pressure on back and pelvis.	» Opioid analgesics: These drugs can dull the pain to help you deal with contractions. » Epidural: this may be requested and set up now if pain feels unmanageable.
Transition phase	Contractions can be extremely intense now, with little rest time in between. Pressure in the lower back and rectum is strong, and you may be exhausted, overwhelmed, and nauseous. You may vomit.	» Controlled breathing: breathing techniques can be used if you feel panicky and out of control: use a combination of more shallow in-breaths and longer exhalations. » Water: you can remain in a birthing pool now if you want to give birth in water.	» Opioid analgesics: These are not an option now; they're avoided close to delivery since they can affect the baby's breathing. » Epidural: can be topped off; you may be encouraged to avoid setting one up now.
2nd stage of labor	Strong, long-lasting contractions and an urge to push down. Strong pressure in the rectum and stinging or burning as the head crowns.	» Breathing: slow and focused exhalations can help you bear down. Panting as the head crowns is important. Pain can feel productive as you push your baby out.	» Epidural: You may still feel the effects of your epidural now.

Q Can I use more than one type of pain relief?

Yes. Most types of natural pain relief can be used alongside medical pain relief. And some types of medical relief can be used together. You may ask for an opioid analgesic at first, then feel that you need an epidural later. That's fine to do. Your doctor should allow you to have opioid analgesics any time up until the point at which you need to start pushing; because the drug crosses into the baby's system and can slow down his breathing, it's better that they aren't administered too close to when the baby is born. If you feel anxious and need a tranquilizer, that can be given with other drugs, but not too close to when the baby will be born, because it also can affect breathing.

Q What kinds of medical pain relief will I be able to have for a home birth?

Women who choose a home birth often want to escape the trappings of a traditional hospital birth: the institutional environment. Maybe they want to sit in a birthing pool at home for pain relief. Maybe they have practiced hypnobirthing or another method that they are comfortable with. Keep in mind that home births are opposed by the American College of Gynecologists and Obstetricians because of the potential for complications, even in low-risk pregnancies. (To be considered for one, you must have a low-risk pregnancy and no preexisting medical conditions.) You can have a medication-free delivery in a hospital, too.



Did you know...

Although home birth limits your access to analgesics, studies show that women who have had a home birth feel more positive about their experience of labor pain. Whether this is because they have little expectation of being able to have pain relief (so have already reached a certain level of acceptance about the pain), because they are more relaxed in their environment, or a combination of the two is unknown. At home and in a hospital, the support of a doctor and a birth partner is beneficial.